

ESOMAC

21096415

ESOMAC

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ESOMAC-20 Gastro Resistant Tablets 20mg

ESOMAC-40 Gastro Resistant Tablets 40mg

Composition

ESOMAC-20

Each film coated tablets contains:
Esomeprazole Magnesium Dihydrate equivalent to
Esomeprazole20 mg
(As enteric coated pellets)
Colour: Iron oxide red and Titanium dioxide

ESOMAC-40

Each film coated tablets contains:
Esomeprazole Magnesium Dihydrate equivalent to
Esomeprazole 40 mg
(As enteric coated pellets)
Colour: Iron oxide red, Iron oxide black, Titanium dioxide & Erythrosine

Dosage Form

Gastro resistant Tablets

Product Description

20 mg: Pink, coloured capsule shaped, Biconvex, Film coated tablets, debossed "20" on one side and plain on other side.
40 mg: Pink, coloured capsule shaped, Biconvex, Film coated tablets, debossed "40" on one side and plain on other side

Pharmacology

Pharmacodynamics

Pharmaceutical group: Drugs for acid-related disorders proton pump inhibitors.

ATC Code: A02B C05

Esomeprazole is the S-isomer of omeprazole and reduces gastric acid secretion through a specific targeted mechanism of action. It is a specific inhibitor of the acid pump in the parietal cell. Both the R- and S-isomer of omeprazole have similar pharmacodynamic activity.

Mechanism of action

Esomeprazole is a weak base and is concentrated and converted to the active form in the highly acidic environment of the secretory canaliculi of the parietal cell, where it inhibits the enzyme H⁺K⁺-ATPase – the acid pump and inhibits both basal and stimulated acid secretion.

Pharmacodynamic effects

After oral dosing with esomeprazole 20 mg and 40 mg the onset of effect occurs within one hour. After repeated administration with 20 mg esomeprazole once daily for five days, mean peak acid output after pentagastrin stimulation is decreased 90% when measured 6–7 hours after dosing on day five.
After five days of oral dosing with 20 mg and 40 mg of esomeprazole, intragastric pH above 4 was maintained for a mean time of 13 hours and 17 hours, respectively over 24 hours in symptomatic GERD patients. The proportion of patients maintaining an intragastric pH above 4 for at least 8, 12 and 16 hours respectively were for esomeprazole 20 mg 76%, 54% and 24%. Corresponding proportions for esomeprazole 40 mg were 97%, 92% and 56%.
Using AUC as a surrogate parameter for plasma concentration, a relationship between inhibition of acid secretion and exposure has been shown. Healing of reflux esophagitis with esomeprazole 40 mg occurs in approximately 78% of patients after four weeks, and in 93% after eight weeks.
One week's treatment with esomeprazole 20 mg b.i.d. and appropriate antibiotics, results in successful eradication of *H. pylori* in approximately 90% of patients.
After eradication treatment for one week, there is no need for subsequent monotherapy with antisecretory drugs for effective ulcer healing and symptom resolution in uncomplicated duodenal ulcers.
During treatment with antisecretory medicinal products, serum gastrin increases in response to the decreased acid secretion. Also CgA increases due to decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours. Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is to allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range
An increased number of ECL cells possibly related to the increased serum gastrin levels, have been observed in both children and adults during long-term treatment with esomeprazole. The findings are considered to be of no clinical significance.
During long-term treatment with antisecretory drugs, gastric glandular cysts have been reported to occur at a somewhat increased frequency. These changes are a physiological consequence of pronounced inhibition of acid secretion, are benign and appear to be reversible.
Decreased gastric acidity due to any means including proton pump inhibitors, increases gastric counts of bacteria normally present in the gastrointestinal tract. Treatment with proton pump inhibitors may lead to slightly increased risk of gastrointestinal infections such as *Salmonella* and *Campylobacter* and, in hospitalised patients, possibly also *Clostridium difficile*.

Paediatric population

In paediatric GERD patients (<1 to 17 years of age) receiving long-term PPI treatment, 61% of the children developed minor degrees of ECL cell hyperplasia with no known clinical significance and with no development of atrophic gastritis or carcinoid tumours.

Pharmacokinetics

Absorption

Esomeprazole is acid labile and is administered orally as enteric-coated granules. *In vivo* conversion to the R-isomer is negligible. Absorption of esomeprazole is rapid, with peak plasma levels occurring approximately 1-2 hours after dose. The absolute bioavailability is 64% after a single dose of 40 mg and increases to 89% after repeated once daily administration. For 20 mg esomeprazole the corresponding values are 50% and 68%, respectively. Food intake both delays and decreases the absorption of esomeprazole although this has no significant influence on the effect of esomeprazole on intragastric acidity.

Distribution

The apparent volume of distribution at steady state in healthy person is approximately 0.22 l/kg body weight. Esomeprazole is 97% plasma protein bound.

Biotransformation

Esomeprazole is completely metabolised by the cytochrome P450 system (CYP). The major part of the metabolism of esomeprazole is dependent on the

polymorphic CYP2C19, responsible for the formation of the hydroxy- and desmethyl metabolites of esomeprazole. The remaining part is dependent on another specific isoform, CYP3A4, responsible for the formation of esomeprazole sulphone, the main metabolite in plasma.

Elimination

The parameters below reflect mainly the pharmacokinetics in individuals with a functional CYP2C19 enzyme, extensive metabolisers.
Total plasma clearance is about 17 l/h after a single dose and about 9 l/h after repeated administration. The plasma elimination half-life is about 1.3 hours after repeated once daily dosing. Esomeprazole is completely eliminated from plasma between doses with no tendency for accumulation during once-daily administration.
The major metabolites of esomeprazole have no effect on gastric acid secretion. Almost 80% of an oral dose of esomeprazole is excreted as metabolites in the urine, the remainder in the faeces. Less than 1% of the parent drug is found in urine.

Linearity/non-linearity

The pharmacokinetics of esomeprazole has been studied in doses up to 40 mg b.i.d. The area under the plasma concentration-time curve increases with repeated administration of esomeprazole. This increase is dose-dependent and results in a more than dose proportional increase in AUC after repeated administration. This time- and dose-dependency is due to a decrease of first pass metabolism and systemic clearance probably caused by an inhibition of the CYP2C19 enzyme by esomeprazole and/or its sulphone metabolite.

Special patient populations

Poor metabolisers

Approximately 2.9 ± 1.5% of the population lack a functional CYP2C19 enzyme and are called poor metabolisers. In these individuals the metabolism of esomeprazole is probably mainly catalysed by CYP3A4. After repeated once daily administration of 40 mg esomeprazole, the mean area under the plasma concentration-time curve was approximately 100% higher in poor metabolisers than in subjects having a functional CYP2C19 enzyme (extensive metabolisers). Mean peak plasma concentrations were increased by about 60%. These findings have no implications for the posology of esomeprazole.

Gender

Following a single dose of 40 mg esomeprazole the mean area under the plasma concentration-time curve is approximately 30% higher in females than in males. No gender difference is seen after repeated once daily administration. These findings have no implications for the posology of esomeprazole.

Hepatic impairment

The metabolism of esomeprazole in patients with mild to moderate liver dysfunction may be impaired. The metabolic rate is decreased in patients with severe liver dysfunction resulting in a doubling of the area under the plasma concentration-time curve of esomeprazole. Therefore, a maximum of 20 mg should not be exceeded in patients with severe dysfunction. Esomeprazole or its major metabolites do not show any tendency to accumulate with once daily dosing.

Renal impairment

No studies have been performed in patients with decreased renal function. Since the kidney is responsible for the excretion of the metabolites of esomeprazole but not for the elimination of the parent compound, the metabolism of esomeprazole is not expected to be changed in patients with impaired renal function.

Elderly

The metabolism of esomeprazole is not significantly changed in elderly subjects (71-80 years of age).

Paediatric population

Adolescents 12-18 years:

Following repeated dose administration of 20 mg and 40 mg esomeprazole, the total exposure (AUC) and the time to reach maximum plasma concentration (t_{max}) in 12 to 18 year-olds was similar to that in adults for both esomeprazole doses.

Indications

Esomeprazole tablets are indicated for:

Gastroesophageal Reflux Disease (GERD)

- Treatment of erosive reflux esophagitis
- Long-term management of patients with healed esophagitis to prevent relapse
- Symptomatic treatment of gastroesophageal reflux disease (GERD)

In combination with appropriate antibacterial therapeutic regimens for the eradication of *Helicobacter pylori* and

- Healing of *Helicobacter pylori* associated duodenal ulcer and
- Prevention of relapse of peptic ulcers in patients with *Helicobacter pylori* associated ulcers.

Patients requiring continued NSAID therapy

- Healing of gastric ulcers associated with NSAID therapy.
- Prevention of gastric and duodenal ulcers associated with NSAID therapy, in patients at risk.

Prolonged treatment after i.v. induced prevention of rebleeding of peptic ulcers
Treatment of Zollinger-Ellison Syndrome

Dosage and Method of Administration

Posology

Adults and adolescents from the age of 12 years

Gastroesophageal Reflux Disease (GERD)

- treatment of erosive reflux esophagitis

40 mg once daily for 4 weeks.
An additional 4 weeks treatment is recommended for patients in whom esophagitis has not healed or who have persistent symptoms.
20 mg once daily
- symptomatic treatment of gastroesophageal reflux disease (GERD)
20 mg once daily in patients without esophagitis. If symptom control has not been achieved after 4 weeks, the patient should be further investigated. Once symptoms have resolved, subsequent symptom control can be achieved using 20 mg once daily. In adults, an on demand regimen taking 20 mg once daily, when needed, can be used. In NSAID treated patients at risk of developing gastric cancer and duodenal ulcers, subsequent symptom treatment using an on demand regimen is not recommended.

Adults

In combination with an appropriate antibacterial therapeutic regimen for the eradication of *Helicobacter pylori* and

- healing of *Helicobacter pylori* associated duodenal ulcer and prevention of relapse of peptic ulcers in patients with *Helicobacter pylori* associated ulcers

20 mg esomeprazole with 1 g amoxicillin and 500 mg clarithromycin, all twice daily for 7 days.

Patients requiring continued NSAID therapy

- healing of gastric ulcers associated with NSAID therapy

The usual dose is 20 mg once daily. The treatment duration is 4-8 weeks.
- prevention of gastric and duodenal ulcers associated with NSAID therapy in patients at risk.
20 mg once daily.

Prolonged treatment after i.v. induced prevention of rebleeding of peptic ulcers.

40 mg once daily for 4 weeks after i.v. induced prevention of rebleeding of peptic ulcers.

Treatment of Zollinger-Ellison Syndrome

The recommended initial dosage is esomeprazole 40 mg twice daily. The dosage should then be individually adjusted and treatment continued as long as clinically indicated. The majority of patients can be controlled on doses between 80 to 160 mg esomeprazole daily. With doses above 80 mg daily, the dose should be divided and given twice daily.

Children below the age of 12 years

Esomeprazole should not be used in children younger than 12 years since no data is available.

Impaired renal function

Dose adjustment is not required in patients with impaired renal function. Due to limited experience in patients with severe renal insufficiency, such patients should be treated with caution. (see Pharmacokinetics).

Impaired hepatic function

Dose adjustment is not required in patients with mild to moderate liver impairment. For patients with severe liver impairment, a maximum dose of 20 mg esomeprazole should not be exceeded (see Pharmacokinetics).

Elderly

Dose adjustment is not required in the elderly.
For Method of Administration see Instruction for Use.

Contraindications

Hypersensitivity to the active substance, to substituted benzimidazoles or to any of the excipients.
Esomeprazole should not be used concomitantly with nelfinavir (see Drug interactions).

Warnings and Precautions

In the presence of any alarm symptom (e.g. significant unintentional weight loss, recurrent vomiting, dysphagia, haematemesis or melena) and when gastric ulcer is suspected or present, malignancy should be excluded, as treatment with esomeprazole may alleviate symptoms and delay diagnosis.

Proton Pump Inhibitor

Long term use (Regular Surveillance)

Patients on long-term treatment (particularly those treated for more than a year) should be kept under regular surveillance.
On demand treatment
Patients on on-demand treatment should be instructed to contact their physician if their symptoms change in character.

Helicobacter pylori eradication

When prescribing esomeprazole for eradication of *Helicobacter pylori*, possible drug interactions for all components in the triple therapy should be considered. Clarithromycin is a potent inhibitor of CYP3A4 and hence contraindications and interactions for clarithromycin should be considered when the triple therapy is used in patients concurrently taking other drugs metabolised via CYP3A4 such as cisapride.

Indications

Gastrointestinal infections

Treatment with proton pump inhibitors may lead to slightly increased risk of gastrointestinal infections such as *Salmonella* and *Campylobacter* (see Pharmacodynamics).

Clostridium Difficile Diarrhea

Published observational studies suggest that PPI therapy may be associated with an increased risk of *Clostridium difficile* associated diarrhea, especially in hospitalized patients. This diagnosis should be considered for diarrhea that does not improve. Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated.

Vitamin B12 Deficiency

Daily treatment with any acid-suppressing medications over a long period of time (e.g., longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy have been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency are observed.

Hypomagnesaemia

Severe hypomagnesaemia has been reported in patients treated with proton pump inhibitors (PPIs) like esomeprazole for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI.
For patients expected to be on prolonged treatment or who take PPIs with digoxin or drugs that may cause hypomagnesaemia (e.g. diuretics), healthcare professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment.

Risk of fracture

Proton pump inhibitors, especially if used in high doses and over long durations (>1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in the elderly or in presence of other recognised risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10-40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of vitamin D and calcium.

Subacute cutaneous lupus erythematosus (SCLE)

Proton pump inhibitors are associated with very infrequent cases of SCLE. If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping esomeprazole. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors.

Combination with other medicinal products

Co-administration of esomeprazole with atazanavir is not recommended (see Drug interactions). If the combination of atazanavir with a proton pump inhibitor is judged unavoidable, close clinical monitoring is recommended in combination with an increase in the dose of atazanavir to 400 mg with 100 mg of ritonavir, esomeprazole 20 mg should not be exceeded.

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SPECIAL INSTRUCTION:
(FOR PACK INSERT)

1) THICKNESS OF FOLDED LEAFLET SHOULD BE 3 TO 4 MM

2) PROOFLESS ARTWORK

3) Red colour will be given to Outline box of artwork
(This is only for illustration and DON'T PRINT)

Page 1 of 3

Esomeprazole is a CYP2C19 inhibitor. When starting or ending treatment with esomeprazole, the potential for interactions with drugs metabolised through CYP2C19 should be considered. An interaction is observed between clopidogrel and esomeprazole (see Drug interactions). The clinical relevance of this interaction is uncertain. As a precaution, concomitant use of esomeprazole and clopidogrel should be discouraged.

When prescribing esomeprazole for on demand therapy, the implications for interactions with other pharmaceuticals, due to fluctuating plasma concentrations of esomeprazole should be considered. (see Drug interactions).

Interference with laboratory tests

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. To avoid this interference, esomeprazole treatment should be stopped for at least 5 days before CgA measurements (see Pharmacodynamics). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

Excipients:

This medicinal product contains sucrose. Patients with rare hereditary problems of fructose intolerance, glucose-galactose malabsorption or sucrase-isomaltase insufficiency should not take this medicine.

Interactions with Other Medicaments

Effects of esomeprazole on the pharmacokinetics of other drugs

Protease inhibitors

Omeprazole has been reported to interact with some protease inhibitors. The clinical importance and the mechanisms behind these reported interactions are not always known. Increased gastric pH during omeprazole treatment may change the absorption of the protease inhibitors. Other possible interaction mechanisms are via inhibition of CYP2C19.

For atazanavir and nelfinavir, decreased serum levels when given together with omeprazole and concomitant administration is not recommended. Co-administration of omeprazole (40 mg once daily) with atazanavir 300 mg/ritonavir 100 mg resulted in a substantial reduction in atazanavir exposure. Increasing the atazanavir dose to 400 mg did not compensate for the impact of omeprazole on atazanavir exposure. The co-administration of omeprazole (20 mg qd) with atazanavir 400 mg/ritonavir 100 mg resulted in a decrease in the atazanavir exposure. Co-administration of omeprazole (40 mg qd) reduced mean nelfinavir AUC, C_{max} and C_{min} and mean AUC, C_{max} and C_{min} for the pharmacologically active metabolite M8. Due to the similar pharmacodynamic effects and pharmacokinetic properties of omeprazole and esomeprazole, concomitant administration with esomeprazole and atazanavir is not recommended (see Warning and precautions) and concomitant administration with esomeprazole and nelfinavir is contraindicated (see Contraindications).

For saquinavir (with concomitant ritonavir), increased serum levels have been reported during concomitant omeprazole treatment (40 mg qd). Treatment with omeprazole 20 mg qd had no effect on the exposure of darunavir (with concomitant ritonavir) and amprenavir (with concomitant ritonavir). Treatment with esomeprazole 20 mg qd had no effect on the exposure of lopinavir (with concomitant ritonavir).

Methotrexate

When given together with PPIs, methotrexate levels have been increased. In high-dose methotrexate administration a temporary withdrawal of esomeprazole may need to be considered.

Tacrolimus

Concomitant administration of esomeprazole has been reported to increase the serum levels of tacrolimus. A reinforced monitoring of tacrolimus concentrations as well as renal function (creatinine clearance) should be performed, and dosage of tacrolimus adjusted if needed.

Medicinal products with pH dependent absorption

Gastric acid suppression during treatment with esomeprazole and other PPIs might decrease or increase the absorption of medicinal products with a gastric pH dependent absorption. As with other medicinal products that decrease intragastric acidity, the absorption of medicinal products such as ketoconazole, itraconazole and efotimib can decrease and the absorption of digoxin can increase during treatment with esomeprazole. Concomitant treatment with omeprazole (20 mg daily) and digoxin increased the bioavailability of digoxin. Digoxin toxicity has been rarely reported. However, caution should be exercised when esomeprazole is given at high doses in elderly patients. Therapeutic drug monitoring of digoxin should then be reinforced.

Medicinal products metabolised by CYP2C19

Esomeprazole inhibits CYP2C19, the major esomeprazole-metabolising enzyme. Thus, when esomeprazole is combined with drugs metabolised by CYP2C19, such as diazepam, citalopram, mirtapramine, domipramine, phenytoin etc., the plasma concentrations of these drugs may be increased and a dose reduction could be needed. This should be considered especially when prescribing esomeprazole for on-demand therapy.

Diazepam

Concomitant administration of 30 mg esomeprazole decreased clearance of the CYP2C19 substrate diazepam.

Phenytoin

Concomitant administration of 40 mg esomeprazole increased trough plasma levels of phenytoin in epileptic patients. It is recommended to monitor the plasma concentrations of phenytoin when treatment with esomeprazole is introduced or withdrawn.

Voriconazole

Omeprazole (40 mg once daily) increased voriconazole (a CYP2C19 substrate) C_{max} and AUC₀₋₁₂.

Cilostazol

Omeprazole as well as esomeprazole act as inhibitors of CYP2C19. Omeprazole, increased C_{max} and AUC for cilostazol.

Cisapride

Concomitant administration of 40 mg esomeprazole increased area under the plasma concentration-time curve (AUC) and prolongelimination half-life (t_{1/2}) but no significant increase in peak plasma levels of cisapride. The slightly prolonged QTc interval observed after administration of cisapride alone, was not further prolonged when cisapride was given in combination with esomeprazole (see Warning and precautions).

Warfarin

Concomitant administration of 40 mg esomeprazole to warfarin showed that coagulation times were within the accepted range. Monitoring is recommended when initiating and ending concomitant esomeprazole treatment during treatment with warfarin or other coumarine derivatives.

Clopidogrel

A pharmacokinetic (PK) pharmacodynamic (PD) interaction between clopidogrel (300 mg loading dose/75 mg daily maintenance dose) and esomeprazole (40 mg p.o.daily) decreased exposure to the active metabolite of clopidogrel and decreased maximum inhibition of (ADP induced) platelet aggregation.

Inconsistent data on the clinical implications of a PK/PD interaction of esomeprazole in terms of major cardiovascular events have been reported. As a precaution concomitant use of clopidogrel should be discouraged.

Investigated medicinal products with no clinically relevant interaction

Amoxicillin and quinidine

Esomeprazole has been shown to have no clinically relevant effects on the pharmacokinetics of amoxicillin or quinidine.

Naproxen or rofecoxib

Concomitant administration of esomeprazole and either naproxen or rofecoxib did not identify any clinically relevant pharmacokinetic interactions during short-term studies.

Effects of other medicinal products on the pharmacokinetics of esomeprazole

Medicinal products which inhibit CYP2C19 and/or CYP3A4

Esomeprazole is metabolised by CYP2C19 and CYP3A4. Concomitant administration of esomeprazole and a CYP3A4 or CYP2C19 inhibitor (such as clarithromycin or voriconazole) increased esomeprazole exposure. A dose adjustment of esomeprazole is not regularly required in either of these situations. However, dose adjustment should be considered in patients with severe hepatic impairment and if long-term treatment is indicated.

Medicinal products which induce CYP2C19 and/or CYP3A4

Drugs known to induce CYP2C19 or CYP3A4 or both (such as rifampicin and St. John's wort) may lead to decreased esomeprazole serum levels by increasing the esomeprazole metabolism.

Paediatric population

Interaction studies have only been performed in adults.

Fertility, pregnancy and lactation

Pregnancy

Clinical data on exposed pregnancies with esomeprazole are insufficient. Esomeprazole do not indicate direct or indirect harmful effects with respect to embryonal/fetal development, pregnancy, parturition or postnatal development. Caution should be exercised when prescribing to pregnant women.

Esomeprazole do not indicate direct or indirect harmful effects with respect to reproductive toxicity.

Lactation

It is not known whether esomeprazole is excreted in human breast milk. There is insufficient information on the effects of esomeprazole in newborns/infants. Esomeprazole should not be used during lactation.

Fertility

Esomeprazole with the racemic mixture omeprazole, given by oral administration do not indicate effects with respect to fertility.

Undesirable Effects

List of adverse reactions

System Organ Class	Frequency	Undesirable Effect
Blood and lymphatic system disorders	Rare	Leukopenia, thrombocytopenia
	Very rare	Agranulocytosis, pancytopenia
	Rare	Hypersensitivity reactions e.g. fever, angioedema and anaphylactic reaction/shock
Immune system disorders	Rare	Hypersensitivity reactions e.g. fever, angioedema and anaphylactic reaction/shock
Infections & infestations		Clostridium difficile associated diarrhoea.
Metabolism and nutrition disorders	Uncommon	Peripheral oedema
	Rare	Hyponatraemia
	Not known	Hypomagnesaemia (see Warning and precautions); severe hypomagnesaemia can correlate with hypocalcaemia. Hypomagnesaemia may also be associated with hypokalaemia.
		Vitamin B12 deficiency
Psychiatric disorders	Uncommon	Insomnia
	Rare	Agitation, confusion, depression
	Very rare	Aggression, hallucinations
Nervous system disorders	Common	Headache
	Uncommon	Dizziness, paraesthesia, somnolence
	Rare	Taste disturbance
Eye disorders	Rare	Blurred vision
Ear and labyrinth disorders	Uncommon	Vertigo
Respiratory, thoracic and mediastinal disorders	Rare	Bronchospasm
Gastrointestinal disorders	Common	Abdominal pain, constipation, diarrhoea, flatulence, nausea/vomiting, fundic gland polyps (benign)
	Uncommon	Dry mouth
	Rare	Stomatitis, gastrointestinal candidiasis
	Not known	Microscopic colitis
	Uncommon	Increased liver enzymes
Hepatobiliary disorders	Rare	Hepatitis with or without jaundice
	Very rare	Hepatic failure, encephalopathy in patients with pre-existing liver disease
	Uncommon	Dermatitis, pruritus, rash, urticaria
Skin and subcutaneous tissue disorders	Rare	Alopecia, photosensitivity
	Very rare	Erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (TEN)
	Not known	Subacute cutaneous lupus erythematosus (see Warning and precautions
	Uncommon	Fracture of the hip, wrist or spine (see Warning and precautions
Musculoskeletal and connective tissue disorders	Rare	Arthralgia, myalgia
	Very rare	Muscular weakness

Renal and urinary disorders	Very rare	Interstitial nephritis; in some patients renal failure has been reported concomitantly.
Reproductive system and breast disorders	Very rare	Gynaecomastia
General disorders and administration site conditions	Rare	Malaise, increased sweating

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via the local reporting system.

Symptoms and Treatment of Overdose

There is very limited experience to date with deliberate overdose. The symptoms described in connection with 280 mg were gastrointestinal symptoms and weakness. Single doses of 80 mg esomeprazole were uneventful. No specific antidote is known. Esomeprazole is extensively plasma protein bound and is therefore not readily dialyzable. As in any case of overdose, treatment should be symptomatic and general supportive measures should be utilized

Effects on ability to drive and use machine

Esomeprazole has minor influence on the ability to drive and use machines. Adverse reactions such as dizziness (uncommon) and blurred vision (rare) has been reported (see Side effects). If affected patients should not drive or use machines.

Instructions for Use

The tablets should be swallowed whole with liquid. The tablets should not be chewed or crushed. For patients who have difficulty in swallowing, the tablets can also be dispersed in half a glass of non-carbonated water. No other liquids should be used as the enteric coating may be dissolved. Stir until the tablets disintegrate and drink the liquid with the pellets immediately or within 30 minutes. Rinse the glass with half a glass of water and drink. The pellets must not be chewed or crushed.

For patients who cannot swallow, the tablets can be dispersed in non-carbonated water and administered through a gastric tube. It is important that the appropriateness of the selected syringe and tube is carefully tested.

Administration through gastric tube

1. Put the tablet into an appropriate syringe and fill the syringe with approximately 25 ml water and approximately 5 ml air. For some tubes, dispersion in 50 ml water is needed to prevent the pellets from clogging the tube.
2. Immediately shake the syringe for approximately 2 minutes to disperse the tablet.
3. Hold the syringe with the tip up and check that the tip has not clogged.
4. Attach the syringe to the tube whilst maintaining the above position.
5. Shake the syringe and position it with the tip pointing down. Immediately inject 5–10 ml into the tube. Invert the syringe after injection and shake (the syringe must be held with the tip pointing up to avoid clogging of the tip).
6. Turn the syringe with the tip down and immediately inject another 5–10 ml into the tube. Repeat this procedure until the syringe is empty.
7. Fill the syringe with 25 ml of water and 5 ml of air and repeat step 5 if necessary to wash down any sediment left in the syringe. For some tubes, 50 ml water is needed.

Incompatibility

Not applicable.

Shelf-Life

24 months

Storage and Handling Instruction

Store below 30°C

No special requirements for disposal.

Packaging Information

Carton containing 3 Alu-Alu blister of 10 Tablets each

Manufactured By

Cipla LTD.
L-139 to L-146
Verna Industrial Estate,
Verna Goa, India

Product Registration Holder

Cipla Malaysia Sdn Bhd
Suite 1101, Amcorp Tower,
Amcorp Trade Centre
18 Persiaran Barat, 46050,
Petaling Jaya, Selangor Malaysia

Last Updated: January 2020

Cipla

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ARTIST PATH	
MATERIAL	ITC paper with perforated tape printed with "Tear here"
NO. OF COLOUR	1
PANTONE	Black-Black
INK DETAILS	Oil based Ink from DIC OR MICRO
LENGTH	472 mm $\pm$ 2 mm
LENGTH AFTER FOLDING	59 mm $\pm$ 2 mm
WIDTH	180 mm $\pm$ 2 mm
WIDTH AFTER FOLDING	30 mm $\pm$ 2 mm
GRAMMAGE	NLT 35 gsm
MATERIAL SPECIFICATION	40 GSM ITC Paper with perforated tape printed with "↑Tear Here↑"
PHARMACODE DIRECTION	31869
TYPE	Booklet
DATE	03-October-2022
ARTWORK PRINT SIZE	Actual
SUPERSEDES/REFERENCE	Supersedes: 21085501
ENGVIEW KEYLINE	NA
INSTRUCTIONS/REMARK	- Any deviation must be brought to the notice of packaging development co-ordinator immediately. - For any clarification, please contact packaging development co-ordinator immediately. - NO CHANGES IN ARTWORK SHOULD BE DONE BY THE PRINTER - The printer should verify the e-proof against the approved artwork before submitting for approval and the e-proof should have printer details.

Finalization Summary			
ACTION	DEPARTMENT	ACTION BY	ACTION ON
Prepared by	Packaging Officer	Vinod Vishwakarma	23-Sep-2022 09:39:00
Uploaded by	Packaging Artist	Vaibhav Sawant	26-Sep-2022 11:47:00
Checked by	Packaging Officer	Vinod Vishwakarma	03-Oct-2022 15:08:00
Checked by	Packaging Approver	Maxi Rodricks	07-Oct-2022 21:33:00
Checked by	Regulatory Affairs	Kavita Gawade	13-Oct-2022 15:39:00
Checked by	Licensing	Monika Lakhani	27-Dec-2022 11:28:00
Approved by	Corporate Quality Assurance	Bina Mehta	28-Dec-2022 13:44:00